HEAD AND NECK & THYROID URGENT SUSPECTED CANCER REFERRAL FORM

NOTE: This form is NOT for use for patients under 16yrs

Patient First Name		**Patient Preferred/ Known Name**			
Patients Last Name		**Gender**		**DOB**	
NHS Number		**Patients Best Contact numbers**			
Patient Email		**Patient happy a text / voice msg be left**			
Patient Address		**Patient aware of USC referral**	**Yes / No**	**PIL re: USC given?**	**Yes / No**
Referrer Name:		**Date of decision to refer:**			
Referrer Background:	**General Practitioner** ☐	**General Dental Practitioner** ☐	**ANP / AHP / Other please enter**		

Referring Practice Details

Practice Code		**Telephone No**	
Address		**Practice Email**	

Area of concern & Clinical Information (please see map on last page for guidance)

Mouth (in front of tonsils) ☐	**Skin of face** ☐ **Lips** ☐	**Nasal Cavity** ☐	**Throat (tonsils & beyond)** ☐	**Unexplained Neck lump** ☐	**Thyroid Lump** ☐

Which team need to see the patient?

OMFS: ☐	**ENT:** ☐

REFERRAL CRITERIA (NG12) -Please tick appropriate boxes

Head and neck cancers - recognition and referral | Health topics A to Z | CKS | NICE

Consider using the HaNC score before referring on a USC ORLHealth.com | HaNC-RC v2

HaNC-RC v.2 Score (if completed) ☐

Please use A&G if worried about H&N cancer in a patient that does not meet these criteria

Unexplained ulceration for more than 3 weeks in the oral cavity or on the lip	☐	Persistent & unexplained (non-intermittent) hoarseness lasting for >3 weeks in patients aged 40 and over. (If other symptoms are present to suggest suspicion of lung cancer, refer via lung cancer guideline)	☐
A lump on the lip or in the oral cavity consistent with oral cancer	☐	Persistent sore throat with associated otalgia, hoarseness or dysphagia	☐
A “red” or “red and white” patch in the oral cavity	☐	Persistent / new unilateral nasal obstruction with bloody discharge	☐
Non-healing extraction sockets (lasting for more than 4 weeks) or suspicious loosening of teeth, where malignancy is suspected	☐	**Unilateral sensorineural hearing loss, tinnitus and dizziness ARE NOT symptoms of head and neck cancer. Please refer patients with these symptoms via the normal channels**	
Persistent and unexplained neck lump (which should be visible or palpable and not a sensation of a lump)	☐		

Reason for referral including history and examination findings (Mandatory Field)

(Expanding space section, for additional information and photo if available)

Investigations should not delay referrals
Do not request an ultrasound as this will be performed in secondary care.
If Thyroid Cancer Suspected please include TFT results in Clinical Information section above; Test Done? ☐

FRAILTY ASSESSMENT: ROCKWOOD PERFORMANCE SCORE

Accurate functional status is needed to assess the most appropriate investigation/s and treatment.
If not known please select a score from the attached link for the Rockwood scale and put in the score box:

https://www.england.nhs.uk/south/wp-content/uploads/sites/6/2022/02/rockwood-frailty-scale_.pdf

SCORE:

CLINICAL BACKGROUND INFORMATION

Anticoagulants / Antiplatelets	☐	**Details/indications:**
Immunosuppressants	☐	**Details/indications:**
Diabetic	☐	**Details/indications:**
Allergies	☐	**Details/indications:**

ADDITIONAL CONSIDERATIONS

Please tick if the answer is yes to any of the questions below and give further information

Transport required?	☐	**If yes, please give further information:**
Language/Hearing difficulties? (interpreter required?)	☐	
Mental capacity assessment required?	☐	
Known safeguarding concerns?	☐	
Mobility requirements (unable to climb on/off bed)?	☐	
Learning disability/on SEMI register	☐	

BACKGROUND INFORMATION

Smoker / Ex Smoker (give pack years if known)		BMI / Weight	
Alcohol / Units per week		Other please specify	

Relevant family history	

MODE OF CONSULTATION THAT PROMPTED REFERRAL	
Virtual consultation	☐
Face to Face consultation	☐

GP Practice Details if different from Referrers			
Practice Code		Telephone No	
Address		Practice Email	

Please specify area of concern on the diagram below



